

Semaglutide Commercial Performance Tracker

Commercial Performance, Competitive Dynamics and Product Mix Analysis in the English GLP-1 Prescribing Market

Executive Summary

This project develops a focused commercial performance tracker for semaglutide within the English GLP-1 prescribing market using publicly available NHS prescribing data. The analysis was designed to replicate a commercial analytics use case in which a pharmaceutical analyst is required to understand how a focal product is performing, how its performance compares with the wider market, and which observable product-level factors are associated with changes in performance.

The project focuses on three analytical questions:

1. How is semaglutide performing in terms of prescription volume and recorded cost?
2. How is semaglutide performing relative to the wider GLP-1 market?
3. What product-mix and recorded-cost dynamics are associated with this performance?

The primary commercial comparison uses **January–June 2026 versus January–June 2025**, while monthly trends are analysed across **July 2024 to June 2026**. This distinction is important because the available dataset begins in July 2024, meaning that full-year 2025 versus full-year 2024 comparisons would not represent equivalent periods.

The analysis finds that semaglutide prescription items increased by **3.55% year-on-year**, while the defined GLP-1 market contracted by **3.48%**. This produced a **7.02 percentage-point growth outperformance** and supported a **5.12 percentage-point increase in market share**, with semaglutide reaching approximately **75.47%** of prescription items within the defined market during the 2026 YTD period.

At the same time, semaglutide recorded Net Ingredient Cost declined by **2.13%**, despite positive prescription-item growth. Average recorded NIC per prescription item declined from approximately **£90.54 to £85.58**, representing a **5.48% year-on-year reduction**. This divergence demonstrates that prescription volume and recorded cost do not necessarily move proportionally and should therefore be monitored separately.

The product-mix analysis also identifies material changes across semaglutide presentations. Injection 1 mg remained the largest presentation group, representing approximately **28.68%** of semaglutide prescription items. However, several other strengths increased their mix contribution while some established tablet presentations declined materially. These shifts

provide additional context for understanding aggregate product performance but should not be interpreted as patient-level switching because the source data is aggregated prescribing data.

The final output is a three-page Power BI dashboard designed using a combined Figma and Power BI workflow. The project demonstrates commercial KPI development, pharmaceutical market analysis, Power Query transformation, DAX, time intelligence, data modelling, data-quality assurance, dashboard design and analytical interpretation.

1. Introduction

Commercial analytics within the pharmaceutical industry requires analysts to move beyond simple reporting of absolute prescription volume. Product performance should be assessed relative to the broader market, historical performance, competitive share, product mix and changes in associated cost measures.

A product may, for example, continue to grow in absolute prescription volume while losing share if the market grows faster. Conversely, a product may strengthen its competitive position even when the total market is contracting. Similarly, changes in recorded prescribing cost may differ substantially from changes in prescription volume.

This project applies these principles to semaglutide within the English GLP-1 prescribing market.

The project was intentionally designed as a more focused commercial tracker than previous portfolio projects. Rather than incorporating geography, disease prevalence or opportunity segmentation, it concentrates on the core measures that would typically support routine product and market performance monitoring.

The analysis therefore emphasises:

- prescription items;
- year-on-year volume growth;
- recorded Net Ingredient Cost;
- NIC per item;
- market size;
- market growth;
- market share;
- market share change;
- product versus market growth;
- contribution to net category change;
- presentation mix; and

- presentation mix change.

This narrower scope enables a deeper focus on commercial interpretation and executive dashboard communication.

2. Business Context and Analytical Question

The project assumes the perspective of a commercial analyst supporting a pharmaceutical GLP-1 franchise in England.

The underlying business requirement is to provide decision-makers with a concise understanding of:

- current semaglutide performance;
- competitive performance relative to other GLP-1 molecules;
- the relationship between product growth and category growth;
- changes in product mix; and
- changes in recorded cost per prescription item.

The central business question is therefore:

How has semaglutide performed within the English GLP-1 prescribing market, and to what extent has its performance been associated with category growth, competitive share change, product mix and changes in recorded cost per item?

This question is divided into three analytical perspectives.

Product Performance

How have semaglutide prescription items and recorded NIC changed over time?

Competitive Performance

How is semaglutide performing relative to the wider defined GLP-1 market and competing molecules?

Performance Drivers

What changes in presentation mix and recorded NIC per item are observable alongside overall semaglutide performance?

The wording deliberately refers to observable associations rather than causal effects. The available aggregated prescribing data does not support formal causal inference.

3. Data Source

The project uses publicly available data from the NHS Business Services Authority English Prescribing Dataset.

The source data records prescribing activity at GP-practice level and contains fields including:

- prescribing month;
- chemical substance;
- presentation;
- prescription items;
- quantity;
- total quantity;
- Net Ingredient Cost;
- Actual Cost;
- presentation identifiers; and
- SNOMED codes.

The original source data is highly granular and substantially larger than required for this project.

Because the analysis does not include geographic segmentation, practice-level rows were aggregated to a monthly presentation-level analytical grain.

This approach reduces model size while preserving the dimensions required for:

- temporal analysis;
- molecule-level comparison;
- product presentation analysis;
- cost analysis; and
- market-share calculations.

4. Analytical Scope

4.1 Time Period

The available analytical period is:

July 2024 – June 2026

However, the primary headline comparison is:

January–June 2026 versus January–June 2025

This choice ensures that the comparison uses equivalent time periods.

Because the dataset begins in July 2024, a comparison of full-year 2025 with full-year 2024 would be misleading because only six months of 2024 are available.

The dashboard therefore uses two different temporal perspectives:

- Jan–Jun 2026 vs Jan–Jun 2025 for headline KPI comparisons;
- Jul 2024–Jun 2026 for monthly historical trends.

4.2 Market Definition

For analytical consistency, the GLP-1 market is defined as the following five molecules:

- Semaglutide
- Dulaglutide
- Liraglutide
- Exenatide
- Lixisenatide

Semaglutide is designated as the focal product.

Maintaining a stable market definition is essential because market size and market share depend on the denominator used.

In this project:

Market Share = Molecule Prescription Items / Total Prescription Items in the Defined GLP-1 Market

The result therefore represents prescribing-volume share within the analytical market definition rather than a broader pharmaceutical market share measure.

5. Data Preparation and Quality Assurance

A significant component of the project involved rebuilding and validating the analytical dataset.

5.1 Initial Data Quality Issue

An initially reused prescribing fact table was found to contain incomplete monthly coverage for the required analytical period.

This issue was identified during quality assurance rather than during dashboard construction.

Continuing with the incomplete fact table would have resulted in distorted:

- monthly trends;
- year-on-year comparisons;

- market-share calculations; and
- cost measures.

The data pipeline was therefore rebuilt from the underlying monthly source files.

5.2 Schema Harmonisation

The raw prescribing files were supplied using more than one schema structure across the analytical period.

Two source groups were therefore created and harmonised.

Differences included variations in:

- column names;
- presentation fields;
- chemical-substance fields;
- casing conventions; and
- identifier formats.

The relevant fields were standardised before the two source groups were appended.

The final analytical fact table contains:

- YearMonth
- ChemicalSubstanceCode
- Molecule
- PresentationCode
- PresentationName
- Quantity
- Items
- TotalQuantity
- NIC
- ActualCost
- SNOMEDCode

5.3 Financial Field Data-Type Issue

A second data-quality issue was identified in the treatment of **NIC** and **Actual Cost**.

During an early Power Query transformation, these fields had been converted to whole-number types. This resulted in loss of decimal precision.

Several initial attempts to correct the values using fixed divisors were rejected because the underlying financial values did not consistently contain the same number of decimal places.

The final solution was to correct the transformation at source level.

NIC and **Actual Cost** were parsed as decimal numeric fields using the appropriate locale before append and aggregation.

This restored realistic financial values and produced sensible relationships between:

- NIC;
- Actual Cost;
- prescription items; and
- NIC per item.

This issue demonstrates the importance of validating analytical meaning rather than relying solely on technically successful transformations.

5.4 Aggregation

The source dataset was aggregated to one row per monthly presentation combination.

The grouping dimensions were:

- YearMonth;
- ChemicalSubstanceCode;
- Molecule;
- PresentationCode;
- PresentationName; and
- SNOMEDCode.

The following fields were aggregated using sums:

- Quantity;
- Items;
- TotalQuantity;
- NIC; and
- ActualCost.

This produced a more efficient analytical model without removing information required for the intended commercial analysis.

5.5 Quality-Assurance Checks

The rebuilt dataset was validated across several dimensions.

Checks included:

- complete monthly coverage;
- expected number of months;
- annual and YTD item totals;
- NIC totals;
- Actual Cost totals;
- NIC per Item;
- presentation-level uniqueness;
- dimension-table uniqueness; and
- previous-year measure behaviour.

The final dataset covered the complete period from July 2024 to June 2026.

This QA process was particularly important because many commercial measures are ratios or comparative measures. Small data-quality issues in base measures can therefore produce larger interpretative errors downstream.

6. Data Model

The Power BI model follows a simplified star-schema approach.

6.1 Fact Table

Fact_Prescribing

The central fact table contains monthly presentation-level prescribing measures.

Key measures include:

- Items
- Quantity
- Total Quantity
- NIC
- Actual Cost

It also contains the keys required to connect to time, chemical-substance and presentation dimensions.

6.2 Calendar Dimension

Dim_Calendar

The calendar dimension contains:

- YearMonth;
- Year;
- MonthNumber;
- MonthName;
- Quarter;
- MonthYear; and
- YearMonthSort.

The YearMonth field is used in the active relationship with the fact table.

MonthYear is sorted by YearMonthSort to ensure that monthly trend visuals appear chronologically.

6.3 Chemical Dimension

Dim_Chemical

The chemical dimension contains:

- ChemicalSubstanceCode;
- Molecule;
- Market; and
- FocalProductFlag.

The Market field identifies the defined GLP-1 category, while FocalProductFlag enables semaglutide-specific DAX measures without hard-coding presentation-level filters.

6.4 Presentation Dimension

Dim_Presentation

The presentation dimension contains:

- PresentationCode;
- PresentationName;
- ChemicalSubstanceCode;
- Molecule;
- SNOMEDCode;
- Formulation;
- Strength; and
- PresentationGroup.

Presentation descriptions were transformed into simplified commercial groupings

Examples include:

- Injection 0.25 mg
- Injection 0.5 mg
- Injection 1 mg
- Tablet 3 mg
- Tablet 7 mg
- Tablet 14 mg

This makes presentation analysis substantially easier to interpret than using raw presentation descriptions.

7. Commercial Measures

The analytical framework begins with a small number of base measures.

7.1 Prescription Items

Prescription Items are used as the primary commercial volume measure.

This is preferable to Quantity for cross-presentation comparison because Quantity units may not be directly comparable across formulations.

However, prescription items should not be interpreted as unique patients.

7.2 Net Ingredient Cost

NIC is used as a recorded prescribing-cost measure.

It should not be interpreted as:

- pharmaceutical manufacturer revenue;
- realised selling price;
- net sales;
- profit; or
- commercial margin.

The dashboard therefore deliberately uses the wording:

Recorded NIC

rather than revenue or sales.

7.3 NIC per Item

NIC per Item is calculated as:

Recorded NIC / Prescription Items

This provides a useful diagnostic measure for evaluating whether changes in recorded cost are moving proportionally with changes in prescription volume.

It should not be interpreted as an average pharmaceutical selling price.

7.4 Growth Measures

Year-on-year growth measures compare the current selected period with the corresponding period one year earlier.

For example:

$$\text{Semaglutide YoY Growth} = (\text{Current Semaglutide Items} - \text{Prior-Year Semaglutide Items}) / \text{Prior-Year Semaglutide Items}$$

This framework is also applied to market items, NIC and NIC per Item.

7.5 Growth Gap

Growth Gap is calculated as:

$$\text{Semaglutide YoY Growth} - \text{GLP-1 Market YoY Growth}$$

The measure helps answer whether the focal product is growing faster or slower than the overall category.

7.6 Share Change

Market share change is measured in percentage points rather than percentage growth.

$$\text{Share Change} = \text{Current Market Share} - \text{Prior-Year Market Share}$$

This avoids confusion between percentage change and percentage-point change.

7.7 Contribution to Net Category Change

Semaglutide contribution to category change is calculated as:

$$\text{Semaglutide Absolute Growth} / \text{GLP-1 Market Absolute Growth}$$

During the 2026 YTD period, the total market declined while semaglutide added prescription volume.

The resulting ratio is therefore negative.

This should not be interpreted as semaglutide negatively contributing to market performance. Instead, it indicates that semaglutide partially offset declines elsewhere in the category while the total category still recorded a net decline.

For this reason, the dashboard labels the KPI:

Contribution to Net Category Change

rather than simply Contribution to Growth.

8. Dashboard Design

The final Power BI dashboard contains three pages.

A combined **Figma + Power BI** workflow was used.

Figma was used primarily to develop:

- the header shell;
- visual identity;
- navigation area;
- spacing structure; and
- static branding elements.

Dynamic content remained native to Power BI.

This included:

- KPI cards;
- charts;
- DAX-driven values;
- navigation buttons; and
- analytical summaries.

This approach maintained flexibility while still providing a more polished visual system than using Power BI formatting alone.

The report uses three navigation tabs:

- Overview
- Competition
- Product Mix

No slicers were added to the final design.

This was a deliberate design decision rather than a limitation.

The report is structured as a fixed 2026 YTD executive snapshot supported by historical trend visuals. Additional slicers would have introduced unnecessary interaction without materially improving the core analytical story.

9. Results

9.1 Page 1 - Semaglutide Commercial Performance

The first dashboard page provides an executive overview of semaglutide performance.

The headline KPIs for Jan–Jun 2026 are:

- Prescription Items: 799K
- Items YoY Growth: +3.55%
- Recorded NIC: £68.4M
- NIC YoY Growth: -2.13%
- Market Share: 75.47%
- Share Change: +5.12 percentage points

The page combines these headline measures with:

- a monthly semaglutide prescription-item trend;
- a semaglutide market-share trend; and
- a comparison between semaglutide growth and total market growth.

Interpretation

Semaglutide prescription items increased by **3.55%** relative to Jan–Jun 2025.

However, the defined GLP-1 category declined by **3.48%** over the same period.

This means semaglutide outperformed the market by approximately:

7.02 percentage points

The stronger product growth translated into an increase in market share of approximately:

5.12 percentage points

This is commercially significant because it demonstrates that the product strengthened its competitive position even in a contracting category.

The historical market-share trend also shows that semaglutide's share increased progressively across much of the available period rather than the headline increase being driven by a single month.

However, the analysis should not be interpreted as demonstrating a causal mechanism behind the share gain. The dashboard identifies the competitive outcome, while later analysis explores associated product-mix and cost dynamics.

9.2 Page 2 - Competitive Performance

The second page shifts the analytical perspective from focal-product performance to competitive market dynamics.

Headline KPIs include:

- GLP-1 Market Items: approximately 1.06M
- Market YoY Growth: -3.48%
- Semaglutide YoY Growth: +3.55%
- Growth Gap: +7.02 percentage points
- Contribution to Net Category Change: -71.73%

The main market-trend chart compares prescription-item trends for:

- Semaglutide;
- Dulaglutide;
- Liraglutide;
- Exenatide; and
- Lixisenatide.

Two additional charts compare:

- current market share by molecule; and
- year-on-year market-share change by molecule.

Interpretation

The market-level analysis reveals that the category decline is not uniform across all products.

Semaglutide recorded positive volume growth while the overall category contracted.

This indicates that declines among other molecules were sufficiently large to outweigh the additional semaglutide volume.

The negative **-71.73% contribution to net category change** therefore requires careful interpretation.

A conventional contribution-to-growth measure is intuitive when both the focal product and the market are growing.

In this case:

- semaglutide absolute growth is positive;
- total category absolute growth is negative.

The resulting negative ratio does not indicate negative semaglutide performance. Instead, it reflects semaglutide acting as an offset to declines elsewhere in the category.

Market-share analysis supports this interpretation.

Semaglutide reached approximately **75.47% market share** in the 2026 YTD period and gained approximately **5.12 percentage points** relative to Jan–Jun 2025.

The molecule-level share-change chart shows that this competitive gain occurred alongside share losses among some competing molecules.

The finding therefore demonstrates the importance of separating:

- category performance; and
- focal-product competitive performance.

A contracting category does not necessarily imply weak performance for every product within it.

9.3 Page 3 - Product Mix and Performance Drivers

The third dashboard page examines product-level dynamics within semaglutide.

Headline measures include:

- Semaglutide Items: 799K
- Semaglutide NIC: £68.4M
- Semaglutide NIC per Item: £85.58
- Largest Presentation: Injection 1 mg
- Largest Presentation Mix: 28.68%

The page includes:

- current semaglutide presentation mix;

- presentation mix change;
- semaglutide NIC per Item trend; and
- volume growth versus recorded NIC growth.

Presentation Mix

Injection 1 mg remained the largest semaglutide presentation group during Jan–Jun 2026.

It represented approximately:

28.68% of semaglutide prescription items

Other important presentation groups included several tablet and injection strengths.

The presentation mix was not static.

For example, some presentations gained material share while others declined.

Observed movements included:

- Injection 0.5 mg increasing materially;
- Injection 0.25 mg increasing;
- some newer strengths appearing without a directly comparable prior-year mix;
- Tablet 14 mg declining substantially; and
- Tablet 7 mg declining substantially.

These findings indicate a meaningful shift in the internal composition of semaglutide prescribing.

However, the data does not support the conclusion that individual patients switched directly from one presentation to another.

The analysis therefore refers to:

presentation mix change

rather than patient switching

Recorded NIC Dynamics

The most important cost-related finding is the divergence between prescription volume and recorded NIC.

Semaglutide prescription items increased:

+3.55% YoY

while recorded NIC declined:

-2.13% YoY

At the same time, average recorded NIC per item declined from approximately:

£90.54 to £85.58

equivalent to:

-5.48% YoY

This divergence demonstrates why volume and recorded cost should not be treated as interchangeable measures.

If only prescription-item growth were analysed, the product would appear to be growing.

If only recorded NIC were analysed, the product would appear to be declining.

The combined view shows that semaglutide experienced positive volume growth alongside a reduction in average recorded NIC per item.

This provides a more nuanced interpretation of product performance.

However, NIC per Item should not be interpreted as a manufacturer-level price measure.

Changes may reflect multiple underlying factors, including:

- presentation composition;
- reimbursement-related factors;
- prescribing mix; and
- other dataset-specific cost dynamics.

The available dataset does not allow the contribution of each factor to be isolated formally.

10. Overall Commercial Interpretation

Taken together, the three dashboard pages present a consistent commercial story.

First, semaglutide strengthened its position within the defined GLP-1 category.

Prescription items increased while the total market contracted, resulting in a substantial positive growth gap and increased market share.

Second, category-level decline was not representative of semaglutide performance.

This distinction is important because evaluating the product only through total market growth would obscure its relative competitive strength.

Third, positive prescription-volume growth did not translate into higher recorded NIC.

The reduction in NIC per Item provides evidence that the relationship between volume and recorded cost changed materially.

Fourth, semaglutide's internal product mix also shifted.

The growth of several presentations alongside declines in others means aggregate product growth cannot be understood solely as uniform growth across all presentations.

The overall commercial picture is therefore characterised by:

- positive semaglutide prescription-item growth;
- contraction in the wider GLP-1 category;
- increased semaglutide market share;
- falling recorded NIC per Item; and
- changing presentation mix.

This demonstrates the value of integrating product, market, cost and mix measures within a single analytical framework.

11. Critical Evaluation

The project provides a commercially useful performance-tracking framework, but several methodological considerations are important.

11.1 Market Definition

Market-share calculations depend heavily on market definition.

The project intentionally uses a stable five-molecule GLP-1 market.

Changing the competitive set would change:

- market size;
- product market share;
- market growth; and
- share change.

The dashboard should therefore be interpreted specifically within its stated analytical market.

11.2 Prescribing Activity versus Commercial Sales

The dataset provides prescribing activity rather than manufacturer commercial sales.

Prescription-item growth may be useful as a proxy for product utilisation trends, but it is not directly equivalent to:

- revenue;
- units sold;
- realised demand;
- manufacturer shipments; or
- profitability.

Commercial interpretation must therefore remain within the scope of the source data.

11.3 Cost Interpretation

NIC is a recorded prescribing-cost measure rather than a manufacturer revenue measure.

Similarly, NIC per Item should not be interpreted as an average realised price.

The dashboard uses cost measures to understand how recorded prescribing expenditure moves relative to prescription volume.

It does not attempt to estimate pharmaceutical revenue.

11.4 Aggregation

Aggregating practice-level prescribing data to monthly presentation level improves model efficiency and is appropriate for the project's scope.

However, the trade-off is that the final model cannot support:

regional analysis;

ICB-level analysis;

practice segmentation; or

geographic commercial opportunity analysis.

This was an intentional scope decision rather than a data-model limitation.

12. Limitations

The analysis has several important limitations.

Prescribing Items Are Not Patients

Prescription items represent prescribing activity and cannot be interpreted as unique patient counts.

One individual may generate multiple prescription items.

NIC Is Not Revenue

Recorded NIC is not equivalent to manufacturer revenue, net sales, profit or realised selling price.

Clinical Indication Is Not Explicitly Identified

Semaglutide may be used across different clinical pathways.

The aggregated prescribing dataset does not reliably identify the clinical indication associated with every prescription record.

The project therefore analyses overall semaglutide prescribing rather than indication-specific commercial performance.

Quantity Is Not Fully Comparable Across Presentations

Different formulations may have different quantity definitions.

Prescription Items are therefore used as the primary cross-product volume measure.

Market Scope Is Analytical

The five-molecule GLP-1 definition is an analytical choice.

The results should not automatically be interpreted as representing every therapy that could be considered part of a broader metabolic or diabetes market.

England Scope

The source dataset reflects English NHS prescribing.

The findings should not be interpreted as representing total UK pharmaceutical consumption.

Historical Coverage

The dataset begins in July 2024.

For this reason, full-year 2025 versus full-year 2024 comparisons are avoided.

No Patient-Level Analysis

The dataset does not provide longitudinal patient-level information.

Presentation-mix changes therefore cannot be interpreted as direct switching behaviour.

No Formal Causal Attribution

The project identifies associations between:

- prescription growth;
- category growth;
- market share;
- presentation mix; and
- recorded NIC per Item.

It does not establish that one observed factor caused another.

13. Dashboard and User-Experience Design

The final dashboard was designed to resemble a concise commercial-performance reporting tool rather than an exploratory analytical application.

Each page uses a consistent structure:

- branded header;
- headline KPI row;
- one primary analytical visual;
- supporting comparative visuals; and
- a concise commercial summary.

Navigation is provided using Power BI page-navigation buttons.

The final design does not include slicers.

This was a deliberate design choice.

Because the dashboard's headline purpose is to communicate a fixed 2026 YTD versus prior-year view, unrestricted slicers could weaken interpretative consistency.

Historical context is instead provided through full-period monthly trend charts.

This design prioritises:

- clarity;
- consistency;
- analytical storytelling;
- low interaction complexity; and
- executive usability.

14. Skills Demonstrated

The project demonstrates practical application of:

- pharmaceutical commercial analytics;
- product-performance analysis;
- market-share analysis;
- competitive benchmarking;
- product-mix analysis;
- time-series analysis;
- Power BI;
- Power Query;
- DAX;
- time intelligence;
- star-schema modelling;
- data-quality assurance;
- financial measure validation;
- dashboard UX design;
- Figma;
- analytical storytelling; and
- critical interpretation of secondary healthcare data.

A particularly important learning outcome was the role of data-quality assurance.

The identification of incomplete month coverage and incorrect cost-field typing occurred before final dashboard publication and required rebuilding part of the data pipeline.

This reinforced the principle that analytical accuracy depends not only on correct visualisation or DAX logic, but also on validating the integrity and semantic meaning of the source data.

15. Conclusion

This project developed a Power BI commercial-performance tracker for semaglutide within a defined English GLP-1 prescribing market.

The analysis finds that semaglutide strengthened its competitive position during Jan–Jun 2026.

Prescription items increased by **3.55% year-on-year**, while the wider defined GLP-1 market contracted by **3.48%**.

This resulted in a **7.02 percentage-point growth outperformance** and a **5.12 percentage-point increase in market share**, bringing semaglutide's market share to approximately **75.47%**.

However, the product's performance cannot be understood through prescription volume alone.

Recorded NIC declined by **2.13%**, while average recorded NIC per item declined by **5.48%**, from approximately **£90.54 to £85.58**.

The product's presentation mix also changed materially, with some strengths increasing their share and others declining.

The project therefore demonstrates that a more complete commercial interpretation requires simultaneous consideration of:

- absolute product volume;
- market growth;
- competitive share;
- product mix; and
- recorded cost dynamics.

More broadly, the project shows how publicly available NHS data can be transformed into a commercially oriented analytical product when combined with rigorous data preparation, clear KPI definitions and careful interpretation of what the data can and cannot represent.

Appendix A — Selected DAX Measures

Semaglutide Items

Semaglutide Items =

```
CALCULATE(  
    [Total Items],  
    Dim_Chemical[FocalProductFlag] = "Yes"  
)
```

Semaglutide Market Share

Semaglutide Market Share =

```
DIVIDE(  
    [Semaglutide Items],  
    [GLP1 Market Items]  
)
```

Share Change

Share Change (pp) =

VAR PreviousShare =

[PY Semaglutide Market Share]

RETURN

IF(

NOT ISBLANK(PreviousShare),

([Semaglutide Market Share] - PreviousShare) * 100

)

Growth Gap

Growth Gap (pp) =

VAR ProductGrowth =

[Semaglutide YoY Growth]

VAR MarketGrowth =

[GLP1 Market YoY Growth]

RETURN

IF(

NOT ISBLANK(ProductGrowth)

&& NOT ISBLANK(MarketGrowth),

(ProductGrowth - MarketGrowth) * 100

)

Semaglutide NIC per Item

Semaglutide NIC per Item =

DIVIDE(

[Semaglutide NIC],

[Semaglutide Items]

)

Presentation Mix

Presentation Mix % =

DIVIDE(

[Presentation Items],

CALCULATE(

[Semaglutide Items],

REMOVEFILTERS(Dim_Presentation)

)

)

Presentation Mix Change

Mix Change (pp) =

VAR PreviousMix =

[PY Presentation Mix %]

RETURN

IF(

NOT ISBLANK(PreviousMix),

([Presentation Mix %] - PreviousMix) * 100

)

Appendix B — Suggested Figures

When formatting the final Word document, the following figures should be inserted:

Figure 1. Power BI Data Model

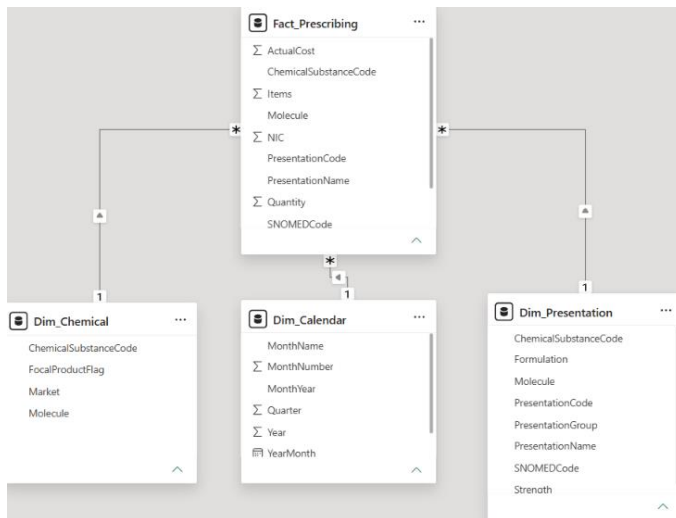


Figure 2. Semaglutide Commercial Performance Dashboard

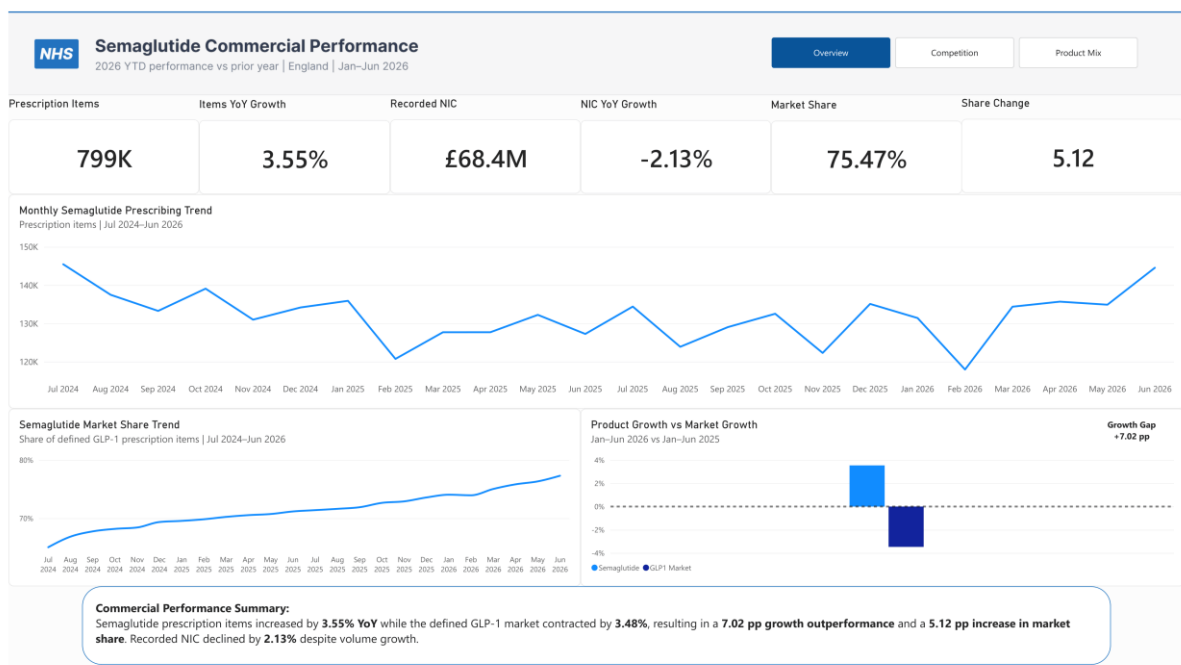


Figure 3. Competitive Performance Dashboard

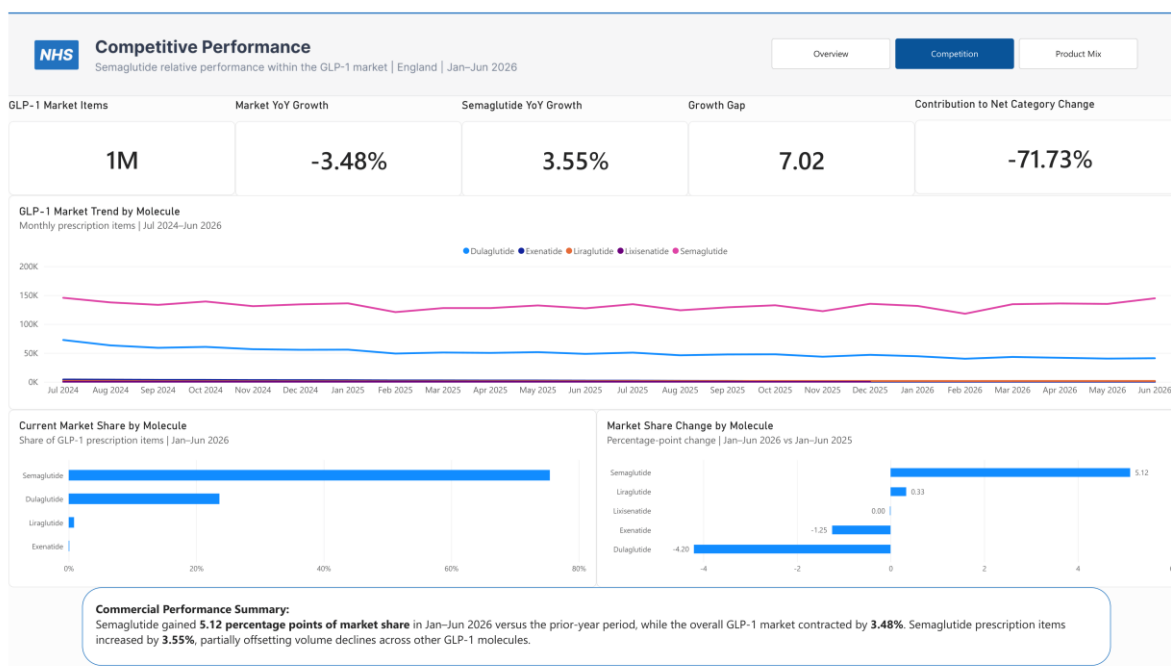
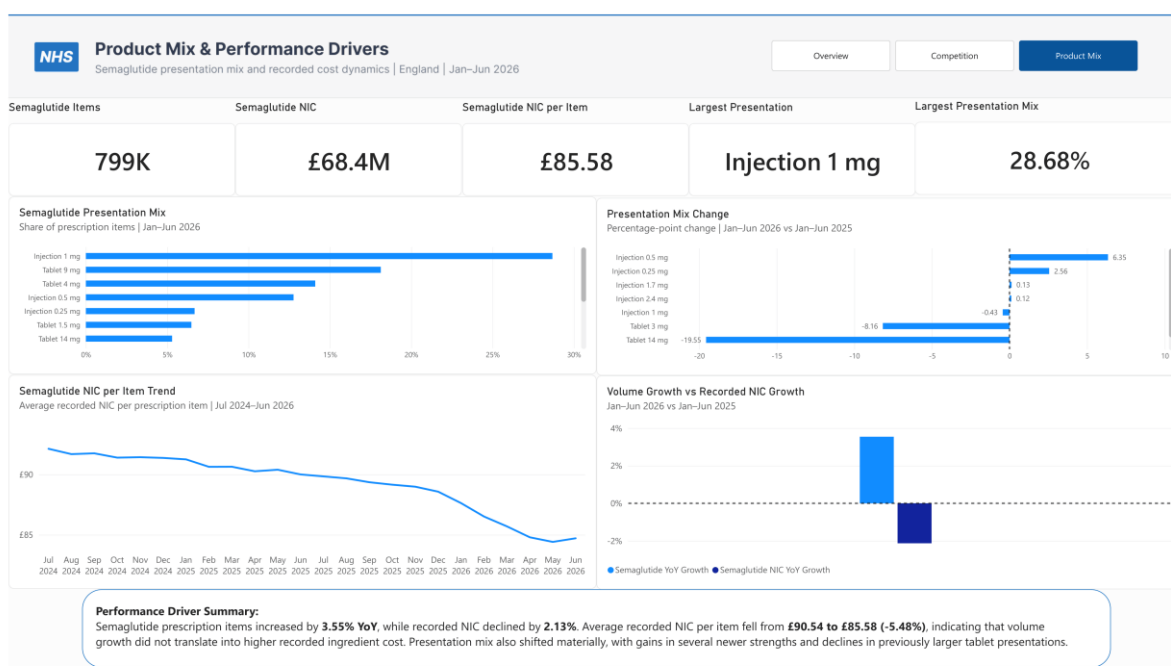


Figure 4. Product Mix & Performance Drivers Dashboard



Data Source

NHS Business Services Authority. **English Prescribing Dataset**.

Publicly available NHS prescribing data were used for independent educational and portfolio analysis.

Disclaimer

This project was developed independently for educational and portfolio purposes.

It is not affiliated with, endorsed by, or produced on behalf of the NHS, any pharmaceutical manufacturer, or any other commercial organisation.

The analysis is intended to demonstrate commercial analytics, data modelling, Power BI, data-quality assurance and dashboard-design capabilities.

It should not be interpreted as clinical, financial, investment or commercial advice.